June 16, 2025

Subject: Response to Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Dear CMS, ASTP/ONC and HHS Teams:

The Digital Medicine Society (DiMe) is a global non-profit and the professional home for digital medicine. Together, we drive broad adoption of digital approaches to advance the practice of medicine to enhance public health. Our mission is to advance the safe, effective, and equitable use of digital approaches to redefine healthcare and improve lives. As CMS, ASTP/ONC and HHS seek input from the public regarding the market of digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure, DiMe appreciates the opportunity to respond to the Request for Information, to inform efforts to lead infrastructure progress to cultivate this market, increasing beneficiary access to effective digital capabilities needed to make informed health decisions, and increasing data availability for all stakeholders contributing to health outcomes.

A core component of our commitment to accelerating the identification and informed adoption of high-quality, trustworthy digital health software products is "The DiMe Seal." With over 350,000 digital health software products available, including more than 30,000 for clinicians and healthcare administrators, the challenge of identifying high-quality, trustworthy options is substantial. The DiMe Seal, developed with an independent Governance Committee, provides a standardized framework to assess digital health software products (DHSPs) across critical domains of **usability, privacy and security, and evidence**. By simplifying this assessment process, the DiMe Seal enables adopters and buyers to confidently select products that are well-made and meet rigorous standards, ultimately improving health outcomes for patients.

We offer the following responses herein specifically from the perspective of the DiMe Seal and referring to sections PC-5, PR-1, TD-1, and VB-1 of the RFI, as requested. The Digital Medicine Society is also submitting a separate response, representing our broader perspective across digital medicine.

PC-5: What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

Encouraging patient and caregiver interest in digital health products requires building fundamental trust, ensuring accessibility, and demonstrating clear value. The DiMe Seal directly addresses these needs by assessing products for quality and trustworthiness, making it easier for patients and caregivers to quickly make confident and informed decisions.

To encourage patient and caregiver interest, CMS and its partners can:

- **Promote awareness of trusted resources:** Actively embrace and endorse independent, private sector, transparent, assessment programs like the DiMe Seal. Leverage accessible lists or directories of awarded products, clearly indicating that these products have met rigorous standards for evidence, privacy and security, and usability. This helps patients and caregivers cut through the noise and identify reliable solutions.
- **Invest in digital health literacy:** Promote and partner with independent organizations to develop and disseminate educational materials (e.g., plain language guides, webinars, community workshops) for Medicare beneficiaries and their caregivers on the identification, benefits and safe use of digital health products. These could include explaining what to look for in a quality app, how to protect their data, how digital tools can complement traditional care, and how to leverage independent, private sector programs like the DiMe Seal to find those digital health products meeting these benefits.
- **Ensure accessibility and usability:** Digital health products must be designed with the diverse needs of Medicare beneficiaries in mind, including those with varying levels of digital literacy, cognitive abilities, technology access and physical impairments. CMS should emphasize and potentially incentivize adherence to robust usability and accessibility standards (e.g., WCAG guidelines) as part of product selection or reimbursement. The DiMe Seal's "Usability" domain explicitly assesses these aspects.

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

CMS has a critical and appropriate role in influencing the adoption of high-quality digital health products, even outside of coverage determinations, by acting as a trusted convener and information disseminator. Given the rapid evolution and sheer volume of digital health products, it is impractical and potentially stifling for CMS to directly "approve" every product for efficacy or quality. Instead, CMS's role should focus on **elevating and endorsing non-regulatory, private sector, robust frameworks for independent, third-party assessment** and creating incentives for the market to reward assessed high-value solutions.

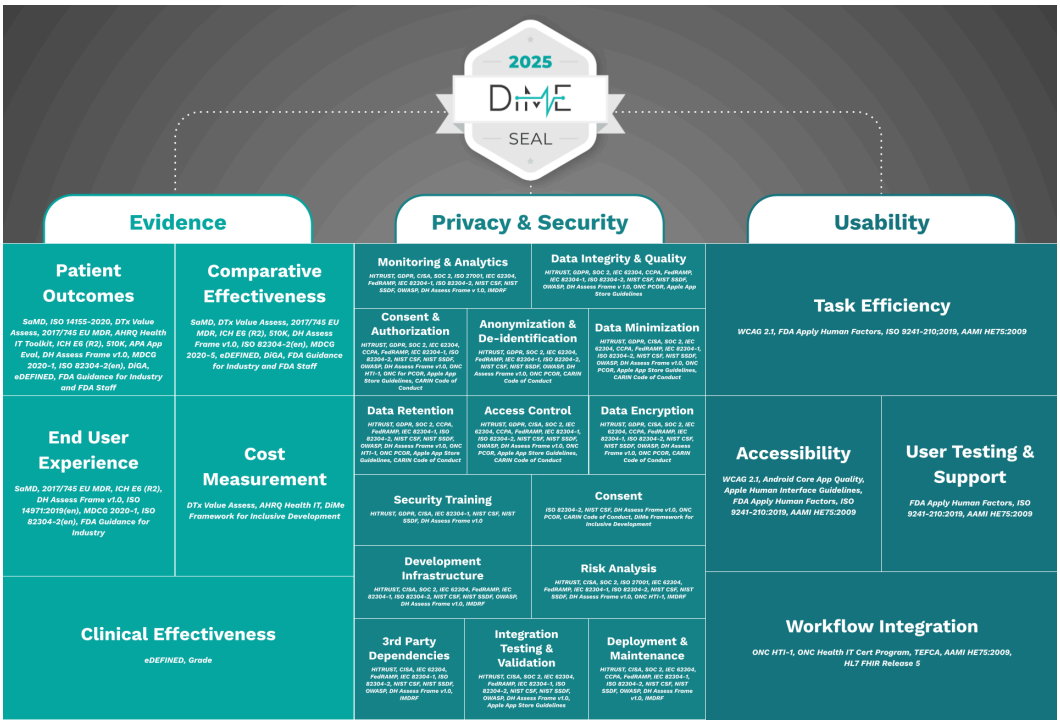
CMS's Role in Review/Approval (beyond coverage):

- **Endorsement of Independent Assessment:** Instead of direct approval, CMS should formally recognize and promote assessments like The DiMe Seal. The DiMe Seal, through its independent Governance Committee and rigorous evaluation process, assesses products based on their **efficacy (evidence generation), quality (usability, privacy & security), and impact on health outcomes (evidenced through clinical studies)**. This leverages private sector expertise and capacity while providing a credible signal of quality to patients, caregivers, and providers.

- **Setting the Standards for "Trustworthy":** CMS can collaborate with organizations like DiMe to define what constitutes a "high-quality, trustworthy" digital health product. This could involve contributing to and endorsing evaluation frameworks that address:
    - **Clinical Evidence:** Clear demonstration of a product's efficacy and impact on health outcomes through appropriately designed studies, stratified by relevant demographic factors (e.g., age, race, socioeconomic status) to ensure generalizability and equitable impact. The DiMe Seal's "Evidence" domain, enhanced by the collaboration with Elevance Health for clinical evidence assessment, is built precisely for this purpose.
    - **Usability and Accessibility:** Products must be intuitive, easy to navigate, and accessible to individuals across the full spectrum of digital literacy and physical/cognitive abilities, especially for Medicare beneficiaries.
    - **Privacy and Security:** Adherence to robust data privacy and security standards (e.g., HIPAA compliance, strong encryption, transparent data use policies) is non-negotiable for building trust.

By focusing on these areas, and elevating the role and work of the private sector such as what we've developed with the DiMe Seal, CMS can empower patients and caregivers to confidently embrace digital health products, fostering a market that prioritizes and rewards high-quality, impactful solutions.

- The DiMe Seal framework incorporates and unifies standards and benchmarks from over 1000 scientific publications, 47 regulatory guidances, 112 standards and regulatory programs and feedback from 150+ industry experts:



PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

To encourage providers, particularly those in rural areas, to leverage assessed digital health products for their patients, CMS and its partners must address existing barriers and clearly articulate the value proposition of these tools. The DiMe Seal provides a crucial mechanism for identifying "awarded" (i.e., independently assessed as high-quality and trustworthy) digital health products, streamlining provider decision-making.

CMS and its partners can encourage adoption by:

- **Promoting and Integrating Third-Party Certifications:** CMS should formally recognize and promote independent, evidence-based assessment programs like The DiMe Seal. By doing so, CMS provides a clear signal to providers that products bearing the DiMe Seal have been rigorously assessed for efficacy, privacy and security and usability. This reduces the burden on individual providers to assess the vast number of available products.
- **Creating Incentives within Value-Based Care Models:** Link the adoption and appropriate use of assessment awarded digital health products to existing or new value-based care initiatives. This could involve quality measures that incentivize the use of digital tools for remote monitoring, patient education, or care coordination, particularly for chronic disease management or preventive care.
- **Providing Education and Training:** Contribute to accessible and practical education and training programs for providers on how to effectively integrate awarded digital health products into their clinical workflows. This should include best practices for selecting, prescribing/recommending, and monitoring patients using these tools. Tailored programs for rural providers, potentially delivered remotely, are essential.
- **Addressing Connectivity and Infrastructure Gaps:** For rural areas, robust broadband infrastructure is foundational. CMS, in collaboration with other federal agencies, should continue to invest in and advocate for policies that expand high-speed internet access in underserved regions, recognizing that digital health adoption is impossible without it.

a. What are the current obstacles?

Current obstacles to provider adoption of digital health products, especially in rural areas, that the DiMe Seal framework addresses, include:

- **Lack of Trust and Information Overload:** Providers face a bewildering array of digital health products, many without robust evidence or transparent claims. It takes approximately 75-150 hours for providers to review a single product. Without a reliable way to differentiate high-quality from low-quality solutions, they are hesitant to recommend them.

- **Integration Challenges:** Many digital health products do not seamlessly integrate with existing Electronic Health Records (EHRs) or clinical workflows, creating additional administrative burden rather than alleviating it. This is a significant pain point for busy clinicians.
- **Reimbursement Uncertainty:** Lack of clear and consistent reimbursement pathways for digital health services or the use of specific digital health products creates financial disincentives for providers.
- **Digital Divide in Rural Areas:** Limited broadband access, insufficient digital literacy among both providers and patients, and a lack of technical support infrastructure can severely hinder adoption in rural settings.
- **Provider Burnout and Time Constraints:** Providers are already stretched thin. Learning new technologies and incorporating them into practice requires time and effort, which can be a significant barrier without clear benefits or supportive infrastructure.
- **Patient Engagement and Adherence:** Even if a provider recommends a digital product, ensuring patient engagement and consistent use can be challenging, impacting the perceived value of the product.
- **Unclear Clinical Utility and Evidence:** Many providers are rightly skeptical of products without strong clinical evidence of efficacy and impact on health outcomes.

The DiMe Seal, through its comprehensive evaluation across these domains, serves as a meta-assessment that integrates and harmonizes these critical standards, offering a single, trustworthy symbol for adopters and buyers. It reduces the burden on individual stakeholders to assess each product against a myriad of individual technical standards.

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To stimulate developer interest in building high-quality digital health products for Medicare beneficiaries and caregivers, CMS must create a more predictable, transparent, and rewarding market environment. Developers are driven by clear market signals, streamlined pathways to adoption, and opportunities for sustainable growth. The DiMe Seal, by defining and recognizing "quality," plays a crucial role in shaping this market.

Short-Term Steps (Next 2 Years):

1. **Clearly Articulate Market Needs and Opportunities:**
    - **Publish Priority Areas:** CMS should clearly delineate specific health challenges or unmet needs within the Medicare beneficiary population that digital health products are well-suited to address (e.g., chronic disease management for specific conditions, caregiver burden, fall prevention, medication adherence). This provides developers with targeted opportunities.

- **Host Developer Challenges/Hackathons:** Sponsor short-term challenges or hackathons focused on these priority areas, offering prizes and opportunities for mentorship or pilot project funding.

2. **Establish a Clear "Trusted Technology" Pathway:**
   - **Formally Endorse Independent Assessments:** CMS should officially recognize and promote credible, independent third-party assessment programs like The DiMe Seal. This provides developers with a clear "quality bar" to aim for and a signal that their investment in robust development, evidence generation, and user-centric design will be recognized. This reduces market ambiguity and accelerates adoption for assessment awarded products.
3. **Pilot Incentive Programs for "Approved" Products:**
   - **Demonstration Projects:** Fund and publicize demonstration projects where healthcare organizations (especially those serving rural or underserved Medicare populations) pilot the use of DiMe Seal-awarded digital health products, showcasing their impact on outcomes and ROI.
   - **"Innovation Waivers" for Value-Based Models:** Explore targeted waivers or flexibilities within existing value-based care models that incentivize the use of independently assessed digital health products.

Longer-Term Steps:

1. **Integrate Assessment into Reimbursement and Quality Frameworks:**
   - **Performance-Based Reimbursement:** Develop long-term strategies to directly link reimbursement for certain digital health-enabled services or products to the product holding a recognized, outcome-focused assessment award (like the DiMe Seal, which emphasizes evidence of impact). This creates a direct financial incentive for developing high-value, high quality, effective products.
   - **Quality Measure Inclusion:** Incorporate the appropriate use of independently assessed digital health products into quality measures for providers within Medicare programs.

By implementing these short-term and longer-term steps, CMS can create a robust and attractive ecosystem that encourages developers to build high-quality, impactful digital health products specifically tailored to the needs of Medicare beneficiaries and their caregivers, ultimately advancing health outcomes across the nation.

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Accountable Care Organizations (ACOs) and participants in the Medicare Shared Savings Program (MSSP) are uniquely positioned to benefit from digital health management and care navigation products due to their focus on population health, care coordination, and value-based outcomes. Incentivizing their effective use requires a multi-faceted approach that addresses both financial drivers and operational barriers. The DiMe Seal serves as a critical enabler by identifying trustworthy, high-quality products that ACOs can confidently adopt.

Incentives to Encourage Leveraging Digital Health Products in APMs:

1. **Integrate Assessed Digital Health Use into Quality Measures:**
    - **Short-Term:** Introduce optional or bonus quality measures within APMs that reward the appropriate adoption and patient engagement with *independently assessed* digital health management and care navigation products (e.g., those with the DiMe Seal) for specific conditions (e.g., chronic disease management, post-discharge follow-up, preventive care). This incentivizes ACOs to use tools that are proven to be high-quality.
    - **Long-Term:** Develop core quality measures that directly reflect improved outcomes or reduced costs attributable to the effective use of digital health products, allowing ACOs flexibility in choosing awarded solutions that best fit their patient population.
2. **Financial Incentives and Flexibilities:**
    - **Shared Savings Attribution:** Clarify how the costs associated with implementing and supporting assessed digital health solutions, including associated personnel (e.g., digital navigators, care coordinators using digital tools), can be factored into ACO expenditures for shared savings calculations.
    - **Innovation Investment Funds:** Create or expand specific funds or grants within APMs dedicated to investing in and deploying assessed digital health technologies, particularly for ACOs serving rural or underserved populations.
    - **Flexibility in Benefit Design:** Allow greater flexibility for ACOs to cover or subsidize patient access to assessed digital health products as part of their care management strategies, especially for products that reduce high-cost utilization (e.g., hospital readmissions).
3. **Streamlined Procurement and Implementation:**
    - **CMS Endorsement of Certification:** By formally endorsing and promoting a standardized, comprehensive assessment like the DiMe Seal, CMS significantly reduces the due diligence burden on ACOs. This signal of "awarded" products streamlines their procurement process, allowing them to confidently select solutions.
    - **Best Practice Sharing:** Facilitate platforms and collaborative networks where ACOs can share best practices, lessons learned, and successful implementation strategies for digital health products, particularly those that have achieved positive outcomes.

By strategically implementing incentives and actively working to dismantle these obstacles, CMS can empower ACOs and MSSP participants to fully harness the transformative potential of high-quality digital health management and care navigation products, ultimately leading to improved health outcomes and more efficient care for Medicare beneficiaries.

In conclusion, The Digital Medicine Society and the DiMe Seal are committed to ensuring that digital health software products are high-quality, trustworthy, and ultimately improve patient outcomes. We believe that by embracing independent assessment processes and integrating them into the broader health technology ecosystem, CMS can effectively accelerate the adoption of valuable digital health solutions, especially within the context of value-based care.

We appreciate the opportunity to provide input on this critical RFI and stand ready to collaborate further with CMS and ASTP/ONC to advance a seamless, secure, and patient-centered digital health infrastructure.

Sincerely,

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